IGNIS CARE LIMITED
30 Pipit Walk Hemel Hempstead HP3 0FR

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Review Sheet

Last Reviewed 5 Mar 2026 | Last Amended 5 Mar 2026 | This policy will be reviewed as needs require or at the following interval: Annual

Business Impact:	MEDIUM — Changes are important, but urgent implementation is not required, incorporate into your existing workflow.
Reason for this Review:	Scheduled review
Changes Made:	Yes
Summary:	This Restrictive Practices Including Restraint and Physical Interventions Policy and Procedure continues to offer guidance around restrictive practices including restraint and physical interventions. It has been reviewed following further personalisation to present information about IGNIS CARE LIMITED, Values, Aims and Objectives. For these changes to reflect in the policy the system details questionnaire will need to be updated.
Relevant Legislation:	• Autism Act 2009 • The Care Act 2014 • Equality Act 2010 • The Health and Social Care Act 2008 (Regulated Activities) (Amendment) Regulations 2012 • Human Rights Act 1998 • Management of Health and Safety at Work Regulations 1999 • Mental Capacity Act 2005 • Mental Capacity Act Code of Practice • Mental Health Act 2007 • Safeguarding Vulnerable Groups Act 2006 • Criminal Law Act (1967) • Criminal Justice Act (2003) • UK GDPR • Data (Use and Access) Act 2025
	• Author: Department of Health, (2022), Deprivation of Liberty Safeguards (DoLS) at a glance [Online] Available from: https://www.scie.org.uk/mca/dols/at-a-glance/ [Accessed: 05/03/2026] • Author: National Institute for Health and Care Excellence, (2015), Violence and Aggression: Short-term management in mental health, health and community settings [Online] Available from: https://www.nice.org.uk/guidance/ng10 [Accessed: 05/03/2026] • Author: Department of Health - Social Care, Local Government and Care Partnership Directorate, (2014), Positive and Proactive Care: Reducing the need for restrictive interventions [Online] Available from: https://www.gov.uk/government/uploads/system/uploads/attachment_da

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Underpinning Knowledge:	ta/file/300293/JRA_DoH_Guidance_on_RP_web_accessible.pdf [Accessed: 05/03/2026] • Author: Care Quality Commission, (2018), Brief Guide: Restraint (physical and mechanical) [Online] Available from: https://www.cqc.org.uk/sites/default/files/20180322_900803_briefguide-restraint_physical_mechanical_v1.pdf [Accessed: 05/03/2026] • Author: Department of Health, (2012), Transforming Care: A national response to Winterbourne View Hospital [Online] Available from: https://www.gov.uk/government/uploads/system/uploads/attachment_data/file/213215/final-report.pdf [Accessed: 05/03/2026] • Author: Department of Health, (2017), Code of practice: Mental Health Act 1983 [Online] Available from: https://www.gov.uk/government/publications/code-of-practice-mental-health-act-1983 [Accessed: 05/03/2026] • Author: HM Government, (2020), Mental Capacity Act Code of Practice [Online] Available from: https://www.gov.uk/government/publications/mental-capacity-act-code-of-practice [Accessed: 05/03/2026] • Author: NATIONAL INSTITUTE FOR HEALTH AND CARE EXCELLENCE, (2015), Challenging Behaviour and Learning Disabilities: Prevention and interventions for people with learning disabilities whose behaviour challenges [Online] Available from: https://www.nice.org.uk/guidance/ng11 [Accessed: 05/03/2026] • Author: GOV.UK, (2019), Reducing the Need for Restraint and Restrictive Intervention [Online] Available from: https://www.gov.uk/government/publications/reducing-the-need-for-restraint-and-restrictive-intervention [Accessed: 05/03/2026] • Author: Local Government Association, The Care Provider Alliance, ADASS, (2016), Promoting less restrictive practice: reducing restrictions tool [Online] Available from: https://www.local.gov.uk/publications/promoting-less-restrictive-practice-reducing-restrictions-tool [Accessed: 05/03/2026] • Author: Department of Health and Social Care, (2019), Reducing the need for restraint and restrictive intervention [Online] Available from: https://www.gov.uk/government/publications/reducing-the-need-for-restraint-and-restrictive-intervention [Accessed: 05/03/2026] • Author: equality human rights, (2019), Human Rights Framework Restraint [Online] Available from: https://www.equalityhumanrights.com/sites/default/files/human-rights-framework-restraint.pdf [Accessed: 05/03/2026] • Author: Resuscitation Council UK, (2024), ILS (Immediate Life Support) Course [Online] Available from: https://www.resus.org.uk/training-courses/adult-life-support/ils-immediate-life-support [Accessed: 05/03/2026] • Author: NHS England, (2015), The Importance of Vital Signs During and After Restrictive Interventions/Manual Restraint [Online] Available from: https://www.england.nhs.uk/wp-content/uploads/2019/12/psa-vital-signs-restrictive-interventions-031115.pdf [Accessed: 05/03/2026] • Author: Gov.UK, (2022), Guidance: Working Definition of Trauma-informed Practice [Online] Available from: https://www.gov.uk/government/publications/working-definition-of-

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	trauma-informed-practice/working-definition-of-trauma-informed-practice [Accessed: 05/03/2026] • Author: Restraint Reduction Network, (2024), Free resources providing guidance on reducing restrictive practices within services [Online] Available from: https://restraintreductionnetwork.org/resources/ [Accessed: 05/03/2026]
Suggested Action:	• Encourage sharing the policy through the use of the QCS App
Equality Impact Assessment:	QCS have undertaken an equality analysis during the review of this policy. This statement is a written record that demonstrates that we have shown due regard to the need to eliminate unlawful discrimination, advance equality of opportunity and foster good relations with respect to the characteristics protected by equality law.

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1. Purpose

1.1 About IGNIS CARE LIMITED

At Ignis Care, we aim to provide safe, stable, and person-centred support for people across a range of needs, enabling them to live fulfilling lives in the community.

Ignis Care delivers support for adults and children with **mental health conditions, learning disabilities and autism, dementia, sensory and physical disabilities**, and other complex needs. We work within **supported living and domiciliary care pathways**, always ensuring tenancy separation in line with the **REACH framework** and the Real Tenancy Test.

Our approach is rooted in **trauma-informed, values-based care**, with a focus on independence, recovery, and preventing crisis escalation. We employ skilled staff who are trained in Positive Behaviour Support (PBS), safeguarding, and specialist interventions, ensuring people are supported safely and compassionately.

Our ambition is to empower each person to achieve their personal goals — whether that means developing daily living skills, sustaining a tenancy, rebuilding relationships, returning to education or employment, or living safely with dignity in later life. We do this through tailored care planning, strong clinical governance, and close collaboration with families, commissioners, housing partners, and NHS/local authority professionals.

What are the values of IGNIS CARE LIMITED?

At Ignis Care, our values guide every decision, every shift, and every interaction. We are committed to:

- **Safety and Skilled Support** – providing high-quality, evidence-based care and support so that every person can achieve their optimum health, wellbeing, and independence.
- **Respect and Dignity** – treating every individual we support, every colleague, and every visitor with respect at all times, and recognising the uniqueness of each person.
- **Human Rights and Citizenship** – upholding the rights of every person to be included, to belong, and to have their voice heard in decisions that affect their life.
- **Choice and Independence** – encouraging informed choices and supporting decision-making, recognising that independence looks different for each person.
- **Privacy and Trust** – safeguarding personal information and respecting privacy in all aspects of daily life.
- **Personal Fulfilment** – enabling people to pursue meaningful goals, activities and relationships that give purpose and joy.
- **Continuous Learning and Improvement** – fostering a culture of reflection, training and feedback so that our staff grow and our service evolves.
- **Trauma-Informed, Person-Centred Care** – understanding people's stories and experiences, and shaping support that responds with compassion, patience and hope.

What are the Aims and Objectives of IGNIS CARE LIMITED?

1.2 Meeting Regulation 18: Staffing

This policy ensures that staffing levels and competencies are sufficient to meet the needs of Service Users, providing high-quality care and support. Ongoing training, professional development, and performance evaluations are conducted to maintain a skilled and capable workforce.

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1.3 Meeting Regulation 17: Good governance

This policy ensures the implementation of strong governance practices, including continuous quality improvement, risk management, and compliance with all regulatory requirements. Effective leadership and accountability structures are in place to support high standards of care/support and service delivery.

1.4 Meeting Regulation 13: Safeguarding Service Users from abuse and improper treatment

This policy aims to safeguard Service Users from abuse and improper treatment by implementing robust procedures for prevention, identification, and response. Staff are trained to recognise signs of abuse and are equipped with clear reporting mechanisms to address any concerns promptly and effectively.

1.5 Meeting Regulation 12: Safe care and treatment

This policy is designed to ensure that all care, support and treatment provided is safe, effective, and in line with best practices and regulatory standards. Risk assessments, staff training, and continuous monitoring are implemented to maintain the highest levels of safety and quality in care and support delivery.

1.6 Meeting Regulation 11: Need for consent

This policy ensures that valid consent is obtained from Service Users for all aspects of their care, support and treatment, in accordance with legal and ethical standards, including the Mental Capacity Act 2005. Informed consent procedures are in place to ensure that Service Users understand and agree to their Care Plans, respecting their autonomy and rights. This policy also ensures that when Service Users lack the capacity to consent, decisions are made in their best interests, following the guidelines set out by the Mental Capacity Act.

1.7 Meeting Regulation 10: Dignity and respect

This policy ensures that all Service User are treated with dignity and respect at all times, promoting a culture of kindness and compassion. Staff are trained to recognise and uphold the intrinsic value of each individual, promoting an environment where Service Users feel valued and respected.

1.8 Meeting Regulation 9: Person-centred care

This policy emphasises the importance of delivering person-centred care, ensuring that all services are tailored to the individual needs, preferences, and values of each Service User. This approach promotes dignity, respect, and independence, enhancing the overall quality of life for Service Users.

1.9 IGNIS CARE LIMITED is committed to upholding human rights and ensuring that least restrictive practices are embedded in all aspects of care and support at all times.

This policy provides a framework to support adult health and social care services in promoting a culture where restrictive practices, including restraint and physical interventions, are used only as a last resort and for the shortest possible duration.

1.10 To create a culture of positive and proactive Support to reduce the need for restrictive interventions and set out mechanisms to ensure accountability to reduce the use of restrictive practices including restraint and physical interventions, as well as effective governance and transparent reporting and monitoring.

1.11 This policy will refer to ‘a behaviour’ of Service Users. In this policy we mean any behaviour or reaction of Service Users that other people may consider to be challenging or concerning.

1.12

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Key Question	Quality Statements
EFFECTIVE	QSE2: Delivering evidence-based care & treatment QSE3: How staff, teams & services work together
EFFECTIVE	QSE6: Consent to care and treatment
SAFE	QSS3: Safeguarding
SAFE	QSS4: Involving people to manage risks QSS5: Safe environments
WELL-LED	QSW1: Shared direction and culture QSW2: Capable, compassionate and inclusive leaders
WELL-LED	QSW5: Governance, management and sustainability

1.13 Relevant Legislation
- Autism Act 2009
- The Care Act 2014
- Equality Act 2010
- The Health and Social Care Act 2008 (Regulated Activities) (Amendment) Regulations 2012
- Human Rights Act 1998
- Management of Health and Safety at Work Regulations 1999
- Mental Capacity Act 2005
- Mental Capacity Act Code of Practice
- Mental Health Act 2007
- Safeguarding Vulnerable Groups Act 2006
- Criminal Law Act (1967)
- Criminal Justice Act (2003)
- UK GDPR
- Data (Use and Access) Act 2025

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2. Scope

2.1 Roles Affected:
- All Staff
- Registered Manager

2.2 People Affected:
- Service Users

2.3 Stakeholders Affected:
- Family

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- Advocates
- Representatives
- Commissioners
- External health professionals
- Legal Representatives

3. Objectives

3.1 Any restrictive practice in use within IGNIS CARE LIMITED is carried out within a clearly defined legal framework, is proportionate to the level of risk, is the least restrictive and only ever used for the minimum duration.

3.2 Protect people's fundamental human rights and promote person-centred, best interest and therapeutic approaches to supporting Service Users when they are distressed.

3.3 Improve the quality of life of those who may, in extreme circumstances, require restrictive practices including restraint and physical interventions to keep them and those supporting them safe.

3.4 Reduce reliance on restrictive practices and interventions by promoting a positive culture and practices that focus on prevention, de-escalation and reflection.

3.5 Increase understanding of the root causes of behaviour and recognise that many behaviours are the result of distress due to unmet needs.

3.6 Where required, focus on the safest and most dignified use of restrictive practices including restraint and physical interventions.

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4. Policy

4.1 The lead for this policy and procedure covering restrictive practice including restraint and physical interventions at IGNIS CARE LIMITED is Mrs Miriam Chengeto Cummings.

It is the responsibility of this lead to ensure that this policy remains up to date and in line with practices carried out at IGNIS CARE LIMITED.

Mrs Miriam Chengeto Cummings has overall responsibility but may delegate some specific responsibilities to other qualified staff members. Delegated responsibilities must be clearly documented, and the assignee must acknowledge and accept the delegation.

Individual care and support plans will detail the specific techniques approved for Service Users following comprehensive risk assessment taking place.

4.2 What is Restraint?

IGNIS CARE LIMITED accepts the Equality and Human Rights Commission's definition of restraint as:

'Restraint is an act carried out with the purpose of restricting an individual's movement, liberty and/or freedom to act independently. Restraint includes chemical, mechanical and physical forms of control, coercion and enforced isolation, which may also be called "restrictive interventions".'

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Legal Framework

There must be a legal framework governing the use of restraint that complies with the following principles:

- The legal framework must include a legal power authorising the use of restraint:
  - In the individual's circumstances
  - For the intended purpose of the restraint

The legal power to restrain may be contained in primary or secondary legislation, or derived from the common law. For this purpose, Section 6 of the Mental Capacity Act 2005 provides IGNIS CARE LIMITED lawful authority for restraint to be used:

- On a person who lacks capacity
- Where it is reasonably believed to be necessary and proportionate to protect them from harm

Unlawful Restraint

It is never lawful to use:

- Restraint with intent to torture, humiliate, distress or degrade someone
- A method of restraining someone that is inherently inhuman or degrading, or which amounts to torture
- Physical force as a means of punishment
- Restraint that unnecessarily humiliates or otherwise subjects a person to serious ill-treatment or conditions that are inhuman or degrading

IGNIS CARE LIMITED recognises and acknowledges the importance of understanding the Service User's perspective and the potential emotional and psychological impact of restraint on them. Staff will be trained to recognise these effects and to use restraint only when absolutely necessary.

4.3 What is Restrictive Practice?

Restrictive practices are a wide range of activities that stop Service Users from doing things that they want to do or that encourage them to do things that they don't want to do.

This term covers a wide range of activities that restrict people and includes:

- Physical intervention/restraint
- Chemical restraint
- Blanket Restrictions
- Environmental restraint
- Mechanical restraint
- Seclusion or enforced isolation
- Long-term segregation
- Coercion
- Surveillance

4.4 NICE guidelines highlight that physical intervention used to forcibly confine a person (also known as manual restraint and the use of mechanical restraint), whether physical or chemical, should not be used in services.

However, manual restraint may be agreed as part of a multidisciplinary decision or agreed at senior management level in exceptional circumstances. Any agreed manual restraint will be for the least time possible and only for someone who lacks capacity where this is deemed as a necessary and proportionate deprivation of liberty.

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IGNIS CARE LIMITED will embed a positive and proactive approach to Support for all Service Users to ensure that any discussions regarding restraint/physical intervention are agreed as part of a best interest decision.

IGNIS CARE LIMITED will continuously review and update practices regarding manual restraint to ensure adherence to the latest NICE guidelines. Regular audits and training updates will be conducted to maintain compliance.

4.5 All staff will understand the actions/interventions that are defined as restrictive practice.

All staff must have positive behaviour support training within learning disability/autism services before undertaking any training that requires restrictive practices including restraint/physical intervention.

In other services, similar training should be undertaken which evidences a positive and proactive approach in supporting challenging, concerning, stressed or distressed behaviours.

All staff must undergo refresher training in positive behaviour support as outlined in IGNIS CARE LIMITED training schedule and will be assessed for competency post-training. Any gaps in knowledge or skills identified during assessments will be addressed promptly through additional training sessions.

4.6 The policy will follow the five principles of the Mental Capacity Act 2005, and any decision made to consider the use of restraint will only be made under legal best interest with relevant parties who can represent the views and wishes of Service Users as well as any other healthcare professionals involved in their Support.

4.7 Care Plans will focus on avoiding or reducing restrictive practices by ensuring that there is a detailed personal history and an up-to-date risk assessment in place for Service Users.

Care Plans and risk assessments will be reviewed and updated in line with IGNIS CARE LIMITED Person-Centred Care and Support Planning Policy and Procedures or immediately following any incident. This ensures that they remain relevant and effective in meeting the Service User's needs.

4.8 Care Plans will contain details of techniques and strategies, such as diversion, prevention, or consideration of allowing the Service User to have their preference used before restrictive practices are considered as an option.

4.9 Before any staff members use restrictive practices including restraint/physical intervention (other than short-term restriction of the Service User's freedom of movement, or in an unforeseen emergency) they will receive appropriate levels of training in line with the Service User's needs, staff roles and responsibilities and will be assessed as competent to carry out any prescribed techniques.

Knowledge and practice will be reviewed on an annual basis, or sooner based on assessment.

4.10 This policy should **ONLY** be implemented in services where the use of restrictive practices, restraint/physical intervention has been agreed at senior management level to support individual Service Users.

Restrictive practices, restraint/physical intervention should only be used when an approved, accredited trainer has delivered the training. An approved list can be found here: Certified organisations - Bild - Association of Certified Training (bildact.org.uk)

4.11 Policy Accessibility

IGNIS CARE LIMITED understands that some Service Users may take in and retain information in different ways. To support full understanding and engagement, this policy is available in accessible formats. The ReciteMe tool has various ways of making this policy accessible by providing this policy in:

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- Audio
- Large print
- Multiple languages

This policy can also be made available in:

- Easy-read versions
- Simple-policy view to reduce navigation and complexity

These options are in place to help Service Users to understand and engage with this policy more easily.

5. Procedure

5.1 Positive Behaviour Support

Positive Behaviour Support (PBS) provides a framework that seeks to understand the context and meaning of behaviour in order to inform the development of supportive environments and skills that can enhance a person's quality of life. Evidence has shown that PBS-based approaches can enhance quality of life and also reduce behaviour, which in turn can lead to a reduction in the use of restrictive practices including restraint/physical intervention (please refer to the Positive Behaviour Support Including Challenging Behaviour Policy and Procedure).

Data on behavioural incidents and outcomes will be collected and analysed to inform and refine PBS plans continually at the IGNIS CARE LIMITED. This data-driven approach ensures that strategies are effective and responsive to the Service User's evolving needs.

All staff should be familiar with the Positive Behaviour Support Including Challenging Behaviour Policy and Procedure at IGNIS CARE LIMITED and have undertaken relevant training on ensuring a positive and proactive approach to managing behaviour.

All staff will undertake training in positive behaviour support/management when supporting Service Users assessed as needing positive behaviour support. PBS is commonly used in learning disability and autism services, but is also effective in supporting any Service User behaviour, including those who are elderly, confused, who may become agitated and may be living with Dementia.

Please refer to the Positive Behaviour Support document in the Forms section of this policy).

5.2 Human Rights

The Human Rights Framework for Restraint covers all forms of restraint covered by the following definition:

- 'Restraint is an act carried out with the purpose of restricting an individual's movement, liberty and/or freedom to act independently. Restraint includes chemical, mechanical and physical forms of control, coercion and enforced isolation, which may also be called 'restrictive interventions'

IGNIS CARE LIMITED accepts that under human rights legislation, basic rights and freedoms belong to every individual without exception.

The policy at IGNIS CARE LIMITED is based on a set core of principles sometimes known as FREDA:

- **F** - Fairness

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- **R** - Respect
- **E** - Equality
- **D** - Dignity
- **A** - Autonomy

These principles are designed to protect Service Users' freedoms and empower them to control how they choose to live their own lives, to take part effectively in decisions made about them which impact upon their rights, and to receive a fair and equal service.

The use of all restrictive practices including restraint/physical intervention must be in line with the principles described in the Human Rights Framework for Restraint produced by the Equality and Human Rights Commission.

Staff at IGNIS CARE LIMITED understand that not all restrictive practice includes the use of force; it can also include acts of interference, for example, the tilting of a recliner chair to make it difficult for Service Users to get out or moving a walking aid out of reach. Even moving a call bell out of reach could be seen as being restrictive as it prevents the Service User from calling for assistance. This may in turn constitute a safeguarding concern as outlined in Section 5.8.

- All acts of restrictive practice must be lawful, proportionate and the least restrictive option available
- It is never lawful to use restrictive practices including restraint/physical intervention to humiliate, degrade or punish any Service User

The best way to avoid restrictive practices is to work preventatively and meet needs before a crisis arises. However, there may be rare occasions when it is necessary to use restrictive practices to prevent harm to Service User or others.

IGNIS CARE LIMITED will ensure that Service Users have appropriate Care Plans in place. This is to ensure that in the event of restrictive practices being identified as needed, the Care Plan contains all the relevant information required that makes the restrictive practice lawful by following recognised procedures.

Care Plans will detail why some Service Users are particularly vulnerable to violations of their human rights. This will include protected characteristics such as age, capacity or because of discrimination related to their protected characteristics as defined by the Equality Act 2010.

Care Plans and risk assessments will detail where Service Users have past trauma, communication barriers, or who have other differences and, as a result, find certain restrictive practices distressing, challenging and harmful and will clearly identify any support required to avoid, reduce and alleviate these processes.

5.3 Trauma Informed Care and Support

It is the aim of IGNIS CARE LIMITED to ensure that all care and support is provided in a way that promotes physical, psychological and emotional safety, respects dignity and individual experiences, and minimises the risk of re-traumatisation.

Trauma-informed principles underpin the approach of IGNIS CARE LIMITED to care and support delivery, staff interactions, service design and organisational culture.

Key Principles

All care and support will be guided by the following principles:

- Safety: Physical, emotional and psychological safety
- Trust and Transparency: Clear, consistent and honest communication
- Choice: Respecting preferences and promoting control

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- Collaboration: Working in partnership with people and those important to them
- Empowerment: Building on strengths and promoting independence
- Cultural Sensitivity and Inclusion: Respecting identity, background and lived experience

Assessment and Planning

Assessments will consider life history, preferences, triggers and coping strategies (where this is appropriate and with the Service User's consent).

Staff Awareness and Training

Where applicable, in line with the training needs analysis at IGNIS CARE LIMITED, staff, including temporary staff, will receive trauma-informed care awareness as part of induction and ongoing training.

Further information can be found in the Trauma Informed Care and Support Policy and Procedure at IGNIS CARE LIMITED

5.4 Capacity and Consent

Staff at IGNIS CARE LIMITED must ensure that they assess the Service User's mental capacity. Consent for the use of any type or method of restrictive practices including restraint/physical intervention must be gained from the Service User unless they lack the mental capacity to make the decision.

Staff must be aware of the following:

- Restrictive practice can only be carried out with the consent of the Service User
- If the Service User has capacity, but does not give consent and there is no risk of harm to other people, then restraint is not acceptable and could result in civil or criminal assault
- If the Service User does not give consent but a restrictive practice is considered necessary to safely manage a situation, minimise the risk, or meet the Service User's needs, then a multidisciplinary discussion must take place in order to discuss and agree alternative strategies
- Where there are concerns over the Service User's capacity to consent to a restrictive practice, then an assessment of the Service User's capacity in relation to that specific issue must be undertaken. A 'best interest' meeting may need to be held in line with the Mental Capacity Act (MCA) 2005 Policy and Procedures
- For Service Users who lack capacity in relation to their behaviour and restrictive practices including restraint/physical intervention, IGNIS CARE LIMITED must have a 'Deprivation of Liberty' agreed which is deemed necessary and proportionate to carry out this level of restriction in specific circumstances

Staff must be aware of the guiding principles of the Mental Capacity Act:

- Principle 1: Assumption of capacity. This means that everyone from the age of 16 is assumed to have mental capacity unless it is established that they lack capacity
- Principle 2: A person must not be assessed as lacking capacity unless all practicable steps have been taken to help them make the decision, without success
- Principle 3: A person is not to be treated as unable to make a decision just because they make an unwise decision
- Principle 4: Anything which is done on behalf of someone who lacks capacity must be done in their best interests
- Principle 5: When taking action or making a decision on behalf of someone who lacks capacity, thought must always be given to whether this could be achieved in a way which is less restrictive of the person's rights and freedom of action

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Advocacy

Where there are significant concerns about any Service User's capacity, an independent mental capacity advocate (IMCA) will be involved to ensure that the Service User's rights and best interests are represented. Where appropriate to do so, Service Users will have access to, and be supported to access, advocacy services in line with the Advocacy Policy and Procedure at IGNIS CARE LIMITED.

Find an Advocate

5.5 Person-centred Practice

IGNIS CARE LIMITED supports the culture of person-centred practice, recognising this as the key to reducing restrictive practice.

IGNIS CARE LIMITED remains fully committed to the reduction of any restrictive practice. All Service Users will receive detailed positive behaviour support (PBS) plans and risk assessments in order to meet Service Users' needs aimed at improving their quality of life and removing the need to implement any restrictive practice. PBS plans will be developed and reviewed in co-production with Service Users, their families, advocates or legal representatives to ensure they reflect the Service User's preferences, needs, and aspirations. This ensures that this policy and any restraint practices will be regularly reviewed with input from Service Users, families, and advocates.

Mrs Miriam Chengeto Cummings will have a process in place for the assessment, monitoring and review of PBS plans in order to understand why Service Users may use certain behaviour to communicate in order to have their needs met by Support Workers.

PBS plans must contain the following:

- Communication needs
- Sensory assessment
- Active support
- Risk management
- Physical health needs
- Trauma informed support
- Consent
- Recovery models
- Any additional relevant information such as how it will be reviewed and monitored for effectiveness
- Working as a multidisciplinary team (MDT), adjustments to PBS plans are made collectively in order to improve support and outcomes for Service Users

Positive behaviour support policies and procedures at IGNIS CARE LIMITED guide staff in the reduction of restrictive behaviours by adopting alternative methods of support. Staff should follow Care Plans and any concerns must be discussed with Mrs Miriam Chengeto Cummings.

Please refer to the Positive Behaviour Support Including Challenging Behaviour Policy and Procedure.

5.6 Before Using Restrictive Practices including Restraint/Physical intervention

Staff must be familiar with the definitions of restrictive practices including restraint/physical intervention.

They will also be clear that, in accordance with the Mental Capacity Act 2005, when considering using any form of restraint with Service Users who lack capacity, the following two

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conditions **must both** be met:

- The Support Worker taking action must reasonably believe that restraint is necessary to prevent harm to the person who lacks capacity, **and**
- The amount and type of restrictive practices including restraint/physical intervention used and the amount of time it lasts must be a proportionate response to the likelihood and seriousness of harm

The following must be assessed:

- The Service User's behaviour
- The Service User's underlying condition and treatment
- The Service User's mental capacity in relation to making decisions about their behaviour which is leading staff to consider using restrictive practices including restraint/physical intervention. This is to include completion of the mental capacity assessment records
- The communication needs of the Service User
- The impact of the use of the type of restrictive practices including restraint/physical intervention on the Service User

It is unlawful to restrain any Service User in a way that deprives them of their liberty unless the procedures set out in the Mental Capacity Act (MCA) 2005 Policy and Procedure and the deprivation of liberty policies and procedures at IGNIS CARE LIMITED are followed.

Staff will ensure that they have considered all other options to ensure the Service User's safety and wellbeing and this is the last resort. Staff can refer to the Restraint/Physical Intervention Flow Chart included in the Forms section of this policy which provides a guide to decision making.

When a decision has been made, a risk assessment will be completed and a Care Plan will be produced. A separate risk assessment for the use of bed rails can be found in Use of Bed Rails Policy and Procedure.

Staff must refer to Positive Behaviour Support Including Challenging Behaviour Policy and Procedure at IGNIS CARE LIMITED to understand how to support and assess Service Users.

Staff should always ask the following questions:

- Is it lawful? – If not, stop it or change it
- Is it necessary? – If not, stop it or change it
- Is it proportionate? – If not, is there a less restrictive way of doing it?

5.7 Care Plans and Risk Assessment

To help protect the interests of Service Users with whom restrictive practices including restraint/physical interventions are used, it is good practice to involve the Service User, and wherever possible, family members, advocates and other relevant legal representatives (e.g. the attorney or deputy for a person who lacks capacity) in planning, monitoring and reviewing how and when they are used. This includes ensuring all reasonable adjustments and that documentation is in a format the Service User understands. If the Service User is not involved, this should be fully documented and justified (refer to the Accessible Information Standard (AIS) Policy and Procedure).

Care Plans must always include clear evidence of health and social needs assessment, and must be created with input from the Service User, their carers, relatives or advocates. This should identify:

- The context in which the Service User's behaviours occur
- Clear, primary, preventative strategies which focus on improvement of quality of life and ensuring that needs are met

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- Secondary preventative strategies which aim to ensure that early signs of anxiety and agitation are recognised and responded to
- Tertiary strategies which may include details of planned restrictive interventions to be used in the safest possible manner, and which should only be used as an absolute last resort

If the Service User is able to be involved in their Care Plan, it is important to capture their views on what support they require when they are displaying challenging, concerning, stressed or distressed behaviours.

Care Plans are developed with a clear focus on cultural competence, ensuring that individual cultural, religious, and linguistic needs are respected and integrated into all aspects of Support delivery.

Reasonable adjustments are identified and documented within the Care Plan to address specific barriers, promoting equitable access and personalised Support tailored to each Service User's unique circumstances.

Refer to the Positive Behaviour Support Including Challenging Behaviour Policy and Procedure for effective Support planning.

As a resource to assist in this preparation, the Local Government Association has provided guidance with Promoting less restrictive practice: Reducing restrictions tool.

Where any restrictive practices have been identified and documented, Mrs Miriam Chengeto Cummings will be responsible for ensuring an appropriate reduction plan is in place.

Plans are reviewed within governance systems at IGNIS CARE LIMITED and using a lessons learnt process.

5.8 De-Escalation, Disengagement and Breakaway Techniques

In line with NICE recommendations, staff should not use restrictive practices including restraint/physical interventions. However, in some circumstances, they may be agreed as part of a multidisciplinary decision and also agreed at senior management level in exceptional circumstances.

Staff have a legal right to defend themselves and a professional duty to protect others from harm.

De-escalation, disengagement and breakaway techniques are suggested for community settings (NICE, 2015). This training must be delivered through an accredited training provider. The training must focus on an awareness of the physical environment, strategies to keep safe, and reduce the likelihood of assault or harm. Further information on how to recognise and manage escalating behaviours appropriately at the earliest opportunity, how to minimise the risk of assault, and how to call for help in an emergency should also be included.

Section 3 of the Criminal Law Act (1967) allows all citizens the right to use force that is reasonable in a threatening situation. However, this must be proportionate to the perceived threat. In situations of high risk, staff must remove themselves from the situation and, if there is immediate risk to life, contact the Police.

Strategies to de-escalate and how to disengage safely should be included in the overall positive behaviour support plan. De-escalation techniques include verbal de-escalation, active listening, offering choices, and creating a calm environment. Staff will receive specialised training in these techniques to ensure they can effectively manage and diffuse potentially challenging situations

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5.9 The Safe and Ethical Use of Restrictive Practices including Restraint/Physical Interventions

- Restrictive interventions should never be used to punish, or for the sole intention of inflicting pain, suffering or humiliation
- There must be a real possibility of harm to the Service User or to staff, the public or others if no action is taken
- The nature of techniques used to restrict must be proportionate to the risk of harm and the seriousness of that harm
- Any action taken to restrict Service User's freedom of movement must be the least restrictive option that will meet the need
- Any restriction should be imposed for no longer than absolutely necessary
- What is done to people, why and with what consequences must be subject to audit and monitoring, and must be open and transparent
- Restrictive interventions should only ever be used as a last resort
- Service Users' families, legal representatives and advocate involvement are essential when reviewing plans for restrictive interventions

When confronted with acute behavioural disturbance, or highly stressed or distressed behaviours, the choice of restrictive practices including restraint/physical interventions must always represent the least restrictive option to meet the immediate need. It should always be informed by the Service User's preference (if known), any particular risks associated with their general health and an appraisal of the immediate environment.

Individual risk factors which suggest a person is at increased risk of physical and/or emotional trauma must be taken into account when applying restrictive practices including restraint/physical interventions.

5.10 Unplanned use of Restraint/Physical Intervention

In line with NICE recommendations, staff must not use manual restraint / physical intervention. In situations of medium risk, staff must consider using de-escalation techniques (Refer to the Positive Behaviour Support Including Challenging Behaviour Policy and Procedure at IGNIS CARE LIMITED or where trained to do so in high risk areas, the breakaway technique).

Restraint/physical intervention must only be used in emergency situations where there is an immediate risk of harm and no alternative de-escalation strategy is effective. Its use should always be a last resort and never as a routine or convenience measure.

Restraint/physical intervention must NOT be used unless this has been agreed as part of a wider multidisciplinary decision involving external professionals.

In situations of high risk, staff must remove themselves from the situation and, if there is immediate risk to life, contact the Police. Unplanned restraint/physical intervention, where staff have not been trained in approved techniques, **must not** be used.

In exceptional circumstances, if an unplanned and documented form of restraint/physical intervention has been used, the following information must be recorded:

- Why restraint/physical intervention took place
- What happened, when and who was involved
- How long the restraint/physical intervention lasted
- What form of restraint/physical intervention was used
- If there was any impact on the wellbeing of the Service User
- If vital signs were taken (as recommended)
- The relevant people have been informed

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This documentation must be completed immediately following the incident to ensure accuracy and transparency. A formal review of the incident must be conducted promptly to evaluate the circumstances, assess the necessity and proportionality of the intervention, identify any learning points, and update Care Plans accordingly to prevent future occurrences.

If there are any incidents of unplanned restraint/physical intervention, staff must complete the appropriate incident or accident form and follow the incident and accident reporting policies at IGNIS CARE LIMITED.

IGNIS CARE LIMITED must ensure that an investigation takes place immediately after the event and the local safeguarding team and the CQC are notified where required to do so.

5.11 Use of Chemical Restraint

IGNIS CARE LIMITED is committed to reducing restrictive practices and preventing the inappropriate use of medication. Chemical restraint, including rapid tranquillisation or the use of psychotropic medication primarily to control behaviour, will only be used as a last resort to prevent serious harm and is not routine care and support.

Any use of chemical restraint must be authorised by Mrs Miriam Chengeto Cummings following multidisciplinary team (MDT) review, or in an emergency, and must align with NICE guidance (NG10), STOMP guidance, as detailed in the Overarching Medicines Management Policy and Procedure at IGNIS CARE LIMITED. Decisions must be supported by informed consent or a best interest decision in accordance with the Mental Capacity Act 2005.

All psychotropic medication use will be clearly prescribed, regularly reviewed, and monitored for effectiveness and side effects. Care/support and behaviour support plans will fully detail the rationale for medication use, identified triggers, non-restrictive alternatives, monitoring arrangements, and plans for reduction or withdrawal where clinically appropriate.

All incidents involving chemical restraint will be recorded, reviewed through governance processes at IGNIS CARE LIMITED, and notified to the CQC where required to do so.

5.12 Planned use of Restraint/Physical Intervention

Staff will be familiar with the definition of restraint/physical intervention and the types of restraint/physical intervention.

Only staff who have had approved, accredited training will be involved in any type of restraint/physical intervention.

They will also be clear that, in accordance with the Mental Capacity Act 2005, when considering using any form of restraint/physical intervention with Service User who lacks capacity, the following conditions must be met:

- The Support Worker taking action must reasonably believe that restraint/physical intervention is necessary to prevent harm to the person who lacks capacity
- The amount and type of restraint/physical intervention used and the amount of time it lasts must be a proportionate response to the likelihood and seriousness of harm, and
- A multidisciplinary approach has been taken and a 'Deprivation of Liberty' is in place to include this restriction practice

The following must be assessed:

- The Service User's behaviour
- The Service User's underlying condition and treatment
- The Service User's mental capacity in relation to making decisions about their behaviour which is leading staff to consider using restraint / physical intervention. This

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is to include completion of the mental capacity assessment records and a best interest meeting
- The communication needs of the Service User
- The impact of the use of the type of restraint/physical intervention on the Service User

It is unlawful to physically restrain any Service User in a way that deprives them of their liberty unless the procedures set out in the Mental Capacity Act (MCA) 2005 Policy and Procedure and the deprivation of liberty policies and procedures at IGNIS CARE LIMITED are followed.

Assessing staff will ensure that they have considered all other options to ensure the Service User's safety and wellbeing and this is the last resort. Staff can refer to the Physical Intervention Flow Chart included in the Forms section of this policy which provides a guide to decision making

5.13 Time Limits, Escalation and Review

Restraint/physical intervention must be applied for the shortest time necessary to manage the immediate risk and must never exceed the time limits set out in the Service User's care and support plans without urgent reassessment.

If the restraint continues beyond this time limit, staff must escalate the situation to Mrs Miriam Chengeto Cummings or the senior on duty immediately. An urgent review will be conducted to evaluate the necessity of ongoing restraint, consider alternative interventions, and ensure the safety and dignity of the Service User.

All instances where time limits are approached or exceeded must be clearly documented and trigger a formal review as outlined in the Service User's care and support plans to prevent recurrence and update plans accordingly. Where applicable, this review may trigger formal MDT review.

5.14 Physical Health Monitoring after Restraint/Physical Intervention

Service Users who have had restraint/physical intervention must have their observations monitored after the incident.

This will include:
- Respiratory rate
- Oxygen saturations
- Temperature
- Systolic blood pressure
- Pulse rate
- Level of consciousness

This should be clearly documented in the Care Plan and if there are any concerns, medical help must be called for.

Injuries Sustained Through the Use of Restraint/Physical Intervention

Any injuries or suspected injuries should be dealt with as soon as it is practical and safe to do so.

The person responsible for managing the incident must check with staff and the Service User to ascertain their wellbeing after the incident and whether they may have any injuries. All injuries must be recorded as part of the post-incident debrief and reporting, and a body map should be completed.

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5.15 Post-incident Review and Support

IGNIS CARE LIMITED will ensure that the process for post-incident restraint/physical intervention reviews will be completed in line with NICE Guidance NG10 and QS154 (NICE, 2015, 2017).

Restrictive practice may trigger traumatic memories for some Service Users and staff should offer support after an incident that has involved a restrictive practice as detailed in Service User Positive Behaviour Support Plans. Other Service Users who may have witnessed the event may also require post-incident support.

Post-incident reviews will involve a structured debriefing process, including individual and team reflections. This ensures that all perspectives are considered, and lessons are learned to improve future practice.

The provision of post-incident support alongside other strategies is a fundamental principle at IGNIS CARE LIMITED as it has been acknowledged the correct support post incident can have the following benefits:

- Rebuild trust
- Assure safety
- Assess injury, physical and emotional wellbeing
- Reflect on the event or practice, learning lessons
- Redirection

Post-incident support must be available after any incident where restrictive practice has been used and after any incident that may have had an impact on Service Users. It should also be available to those Support Workers and others who may have been involved or witnessed the incident.

Service Users with cognitive and/or communication impairments may need to be helped to engage in this process, for example, by the use of simplified language or visual imagery. Other people may not be able to be involved due to the nature of their impairment.

If the Service User wishes to raise a formal concern or complaint, they should be reminded how to access the local complaints procedure of IGNIS CARE LIMITED. Staff or Service Users who have sustained injuries or have been subjected to a serious assault should be supported and consideration should be given to involve the Police.

Good governance at IGNIS CARE LIMITED includes a blame-free approach, lessons learnt and continuous improvements aimed at improving outcomes for Service Users.

IGNIS CARE LIMITED will offer debriefing to Support Workers following any restrictive practices. It will also be offered to discuss issues of concern through team meetings at IGNIS CARE LIMITED, supervision and the appraisal process.

5.16 Post-Incident Debrief

IGNIS CARE LIMITED must ensure that where appropriate, lessons are learned when incidents occur where restrictive interventions, including restraint/physical intervention, have had to be used.

Mrs Miriam Chengeto Cummings or a delegated other should carry out a post-incident analysis (refer to the Forms section of this policy) with all staff involved as soon as possible after the incident, ideally before staff finish their shift.

Individual staff may also require a separate review due to sustaining injuries, psychological distress or anxiety around the incident. Whilst all staff should attend the group post analysis,

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staff should also be encouraged to have an individual reflection to raise any specific concerns that may not be appropriate to do so in a group forum.

The aims of post-incident reviews are to:

- Evaluate the physical and emotional impact on all individuals involved (including any witnesses)
- Identify if there is a need, and if so, provide counselling or support for any trauma that might have resulted
- Help Service Users and staff to identify what led to the incident and what could have been done differently
- Determine whether alternatives, including less restrictive interventions, were considered
- Determine whether service barriers or constraints make it difficult to avoid the same course of action in future
- Where appropriate, recommend changes to the service's philosophy, policies, care environment, treatment approaches, staff education and training
- Where appropriate, avoid a similar incident happening on another occasion

All post-incident reviews should be analysed by Mrs Miriam Chengeto Cummings, which should form part of the overall governance and oversight of restrictive practices in the service and within IGNIS CARE LIMITED.

5.17 Recording and Reporting

Mrs Miriam Chengeto Cummings will monitor and maintain a register of all incidents of restraint/physical intervention. This register will be used to review practice and allow for the opportunity to reduce and eliminate the need. This will also be reviewed as part of overall governance at IGNIS CARE LIMITED.

Mrs Miriam Chengeto Cummings will oversee and ensure that for any Service User having any form of restraint/physical intervention or restrictive practice, that the necessary procedures and authorisation, as detailed within this policy, are in place.

Following any occasion where restrictive practices are used, whether planned or unplanned, a full record should be made. This should be recorded as soon as practicable (and always within 24 hours of the incident). The record should detail:

- The names of the staff and people involved
- The reason for using the specific type of restrictive practices used (rather than an alternative less restrictive strategy)
- The type of restrictive practice employed
- The date and the duration of the restrictive practice
- Whether the Service User or anyone else experienced injury or distress (a body map should also detail any injuries)
- What action was taken

5.18 Safeguarding and Notifications

Where the use of restrictive practice or restraint/physical intervention gives rise to safeguarding concerns, a safeguarding referral will be made promptly and without delay, in line with The relevant Local Authority varies depending on the service user's home location. The correct Local Authority is recorded in the service user's Care Plan and PASS profile. safeguarding protocols. This includes any incident involving suspected abuse, neglect, excessive, inappropriate, or unauthorised use of restraint, or where the Service User has experienced harm or is at risk of harm.

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Families, commissioners, legal representatives and health professionals involved in the Service User's Support may also be required to be informed according to individual Care Plans.

All actions taken will prioritise the Service User's safety, rights, dignity, and wellbeing.

IGNIS CARE LIMITED will act in accordance with the duty of candour requirements by being open and transparent when a notifiable safety incident occurs in relation to restrictive practice or restraint. This includes informing the Service User and/or their representative, offering an apology where appropriate, providing a clear explanation of what is known about the incident, and outlining the actions taken to mitigate harm and prevent recurrence.

Duty of candour notifications and records will be completed in line with regulatory guidance and the policies and procedures at IGNIS CARE LIMITED.

5.19 Restraint/Physical Intervention Reduction

IGNIS CARE LIMITED will have restrictive intervention reduction programmes based on the principles of effective leadership, data informed practice, workforce development, the use of specific restrictive intervention reduction tools, Service User empowerment and a commitment to effective models of post-incident review.

To support reductions, IGNIS CARE LIMITED will:

- Assess, implement and use environmental adjustments and sensory interventions to reduce distress
- Regularly review restrictive practices reduction targets with clear leadership accountability
- Use analysis, data analytics and technology to identify patterns of restraint and inform quality improvement

Restrictive intervention reduction programmes will be reviewed on an ongoing basis. As a minimum, there must be evidence of at least an annual, full evidence-based review of control measures leading to revision and update of corporate action plans. All restrictive intervention reduction programmes and evidence of associated reviews must be made available for inspection for the regulators.

Any Service User with a behaviour support plan advocating the use of restrictive interventions should have clear, proactive strategies including details of primary and secondary preventative strategies (refer to the Positive Behaviour Support Including Challenging Behaviour Policy and Procedure).

There must be assurance mechanisms in place which routinely examine the quality of training provided to staff about positive behavioural support, de-escalation and the use of restrictive interventions. IGNIS CARE LIMITED will utilise technology and data analytics to monitor the use of restrictive practices. This will help identify patterns and trends, enabling targeted interventions to reduce reliance on such practices.

There will be arrangements for staff with differing degrees of specialism and seniority to maintain the competence associated with their role (i.e. the competencies required to deliver an effective behaviour support plan are qualitatively and quantitatively different than those required by a specialist practitioner who undertakes complex assessments and devises behaviour support plans).

IGNIS CARE LIMITED will acknowledge and seek to minimise the risks associated with any restrictive interventions taught to staff.

Services must maintain accurate information that allows them to readily identify which Service Users have behaviour support plans that include the use of restrictive interventions as tertiary

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strategies.

Reviews of the quality of design and application of all positive behaviour support plans will be included in the internal audit of IGNIS CARE LIMITED and will inform organisational increased behaviour support planning and restrictive intervention reduction strategies.

IGNIS CARE LIMITED will publish a public, annually updated, accessible report on their increased behaviour support planning and restrictive intervention reduction, which outlines the training strategy, techniques used (how often) and reasons why, whether any significant injuries resulted, and details of ongoing strategies for bringing about reductions in the use of restrictive interventions.

5.20 Audit and Monitoring

Mrs Miriam Chengeto Cummings will monitor and maintain a register of restraint/physical Intervention used in IGNIS CARE LIMITED. This register will be used to review practice and allow for the opportunity to reduce and eliminate the need.

Mrs Miriam Chengeto Cummings will oversee and ensure that any Service User having any form of restraint or restrictive practice in place has the necessary procedures followed as detailed within this policy.

5.21 Good Governance and Corporate Accountability

IGNIS CARE LIMITED will ensure that robust governance oversight, monitoring and regular review are in place where Service Users who are exposed to restrictive interventions have access to high-quality behaviour support plans that are designed, implemented and reviewed by staff with the necessary skills, and that restrictive interventions are undertaken lawfully. IGNIS CARE LIMITED will ensure that its effective governance frameworks are founded on transparency and accountability.

Details of how the senior management level (or equivalent) authorisation and approval of any restrictive interventions taught to their staff and used in practice will be available.

IGNIS CARE LIMITED will ensure that there is an identified director or equivalent who takes a lead responsibility for restrictive intervention reduction programmes. Service Users and families will be informed of who this is.

The senior management team who authorises the use of restrictive interventions in the service should also undertake appropriate training in the use of PBS and restraint/physical interventions to ensure they are fully aware of the techniques their staff are being trained in.

5.22 Learning and Development

The Restraint Reduction Network Training Standards apply to all training that has a restrictive intervention component. Before delivering any training, IGNIS CARE LIMITED will ensure that the accredited, certified training provider carries out a training needs analysis to obtain as much information as possible about the needs and characteristics of staff and Service Users. This will ensure that any restraint/physical intervention is appropriate, proportional, meets identified needs, and that any elevated risks are highlighted and adjustments made where needed. De-escalation and breakaway techniques may be the appropriate training required without the need for physical restraint training.

These standards will ensure that training is directly related and proportional to the needs of individual Service Users and that training is delivered by competent and experienced training staff who can evidence knowledge and skills that go beyond the application of physical restraint or other restrictive interventions.

In addition to improving training and practice, the standards will:

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- Protect Service Users' fundamental human rights and promote person-centred, best interest and therapeutic approaches to supporting Service Users when they are distressed
- Improve the quality of life of Service Users being restrained and those supporting them
- Reduce reliance on restrictive practices by promoting positive culture and practice that focuses on prevention, de-escalation and reflective practice
- Increase understanding of the root causes of the Service User's behaviour and recognition that many behaviours are the result of distress due to unmet needs
- In exceptional circumstances, and only in agreement with external professionals, focus on the safest and most dignified use of restrictive practices including restraint/physical interventions

Mrs Miriam Chengeto Cummings must ensure all staff have access to positive behaviour support (PBS) training which underpins the support of Service Users who may become distressed.

IGNIS CARE LIMITED will ensure staff will have access to ongoing professional development opportunities and the latest research and best practices in the field. This ensures that its practices remain aligned to up-to-date guidance and best practice.

In addition, training should be in place which supports individual needs such as autism, brain injury and mental health conditions.

Any training in restraint/physical intervention must only be delivered after all relevant staff have completed an appropriate level of immediate life support training (including required refresher training). This should be in accordance with the guidelines of the UK Resuscitation Council for immediate life support.

The accredited training provider or in-house trainer must ensure that the training identifies:

- Who the training is intended for
- Aims, objectives and learning outcomes for each programme
- Training methods
- Timings assessment methods
- Rationale that justifies the inclusion of each restrictive intervention in the programme. This may be in any format, but must include the following as a minimum:
    - RRN Training Standards - Name and description of restrictive intervention (diagram or photo)
    - Rationale for use (why and in what situation?)
    - How the intervention will be taught to staff and how competence will be tested
    - General safety issues for staff during teaching and practice
    - Any person-specific safety issues for staff during teaching and practice (where information has been provided, and adjustments need to be made)
    - Any issues that may compromise the fidelity of the technique between the taught version in the classroom and its application in practice. This must include a description of how any identified issues may compromise both safety and effectiveness
    - General safety guidelines, supporting those restrictive interventions authorised for use at population level
    - Person-centred safety guidelines, principles of trauma-informed practice, supporting personalised restrictive interventions
    - A statement that the restrictive intervention must be used as taught and not modified, unless authorised by the accredited training provider

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Training Providers Must Ensure:

- That any form of mechanical restraint/physical intervention that they are requested to teach the use of has been agreed at IGNIS CARE LIMITED senior management level
- Mechanical restraint must only be considered for use in exceptional circumstances in specific settings and under very specific circumstances
- Sharing of NICE (NG10) 2015 guidance, that taking Service Users to the floor during manual restraint/physical intervention should be avoided, but that if it is necessary, the supine (face up) position should be used in preference to the prone (face down) position:
    - This position can, and has, caused death after as little as 10 minutes, by causing a cardiac event. Consistent implementation of these recommendations will save lives, improve safety and minimise distress for all involved

5.23 Care and Support at IGNIS CARE LIMITED

IGNIS CARE LIMITED provides care and support to Service Users with a range of needs in a person-centred, safe, and lawful way.

All staff must follow the guidance within this policy and the Service User's Care Plan, ensuring that assessed needs, reasonable adjustments and individual preferences are met.

- Every Service User is treated equally and with dignity and respect
- Care and support are tailored to individual needs, preferences and desired outcomes
- Staff follow legal, regulatory, and professional guidance at all times
- Person-centred approaches are used to promote independence, choice, and wellbeing

To support this approach, staff will also follow the policies and procedures below where applicable:

- Person-Centred Care and Supporting Planning
- Safeguarding Adults
- Raising Concerns, Freedom to Speak Up and Whistleblowing
- Mental Capacity Act (MCA) 2005
- Deprivation Of Liberty in Community Settings
- Consent to Care, Support and Treatment
- Equality, Diversity and Human Rights
- Overarching Medicines Management
- Positive Behaviour Support Including Challenging Behaviour
- Restrictive Practices Including Restraint and Physical Interventions
- Sex, Sexuality and Relationships

This list is not exhaustive and there will be additional policies and procedures in place to support specific Service User needs. Staff must seek clarification from their line manager or the Registered Manager, Mrs Miriam Chengeto Cummings, if there is any uncertainty.

Staff supporting any specialist area of need will receive appropriate induction and training. They will complete competency assessments, where required, to meet the needs of Service Users as outlined in the Training Policy and Procedure at IGNIS CARE LIMITED.

To ensure Service Users receive person-centred care and support from IGNIS CARE LIMITED, staff must adhere to the following :

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6. Definitions

6.1 Restrictive Practice

- Restrictive practices are defined as: 'Making someone do something they don't want to do or stopping someone doing something they want to do.' (Skills for Care and Skills for Health, 2014, p. 9)
- Restricting practice risks a breach of the following human rights:
  - The right to freedom from torture, inhuman and degrading treatment
  - The right to liberty and security
  - The right to respect for private and family life, home and correspondence
- Examples of restrictive practice include:
  - Use of blanket rules (routine locking of doors, observation levels, use of seclusion, restraining an aggressive Service User, sedation with medication)

6.2 Restrictive Interventions

- This is a specific set of interventions. They are deliberate acts (that could be seen to restrict a {Service_user_text}'s movement, liberty or freedom to act independently) placed on a person to:
  - Take immediate control of a dangerous situation (where there is the risk of harm to the person or others if no action is taken) and
  - End or reduce significantly the danger to the person or others
- Examples could include:
  - Physical interventions and restraint (including mechanical restraint)
  - Seclusion
  - Rapid tranquilisation
  - Personal and other searches
  - Enhanced supervision
  - Withholding of information or equipment
  - Blanket restrictions

6.3 Restraint

- 'The use or threat of force to help do an act which the person resists, or the restriction of the person's liberty of movement, whether or not they resist. Restraint may only be used where it is necessary to protect the person from harm and is proportionate to the risk of harm.' (Mental Capacity Act 2005)
- Any restrictive intervention must be justified both ethically and legally. It must only be:
  - Imposed if necessary to prevent serious harm
  - The least restrictive option
  - Approved using the correct authorisations
- Deprivation of liberty is a form of restraint and it means to prevent someone from exercising their rights and freedoms

6.4 Mechanical Restraint

- Any restrictive device, for example, a wheelchair lap belt, seat belt, bucket chair that restricts a person's movement

6.5 Seclusion

- Seclusion refers to the supervised confinement and isolation of the Service User, away from other Service Users, in an area from which the Service User is prevented from leaving, where it is of immediate necessity for the purpose of the containment of severe

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behavioural disturbance which is likely to cause harm to others (Mental Health Act 1983 Code of Practice 2015, 26.103)

6.6 Blanket Restrictions

- Blanket restrictions are rules or limitations applied to all individuals within a service or setting, regardless of their individual circumstances, capacity, or risk. They are not based on personalised assessment and can unnecessarily restrict freedom, choice, or autonomy

7. Key Facts - Professionals

Professionals providing this service should be aware of the following:

- The prevention of the use of any form of restrictive practices including restraint/physical intervention is the main objective of Support
- The Care Plan must detail specific behaviour, de-escalation techniques, alternative approaches to reduce the need for restraint/physical intervention, and clearly state that restraint/physical intervention must only be used as a 'last resort'
- The use of any restrictive practices including restraint/physical intervention must be accurately recorded and shared with the management of IGNIS CARE LIMITED and, if ongoing, be subject to regular review
- When restraint/physical intervention is used, this must be used for the shortest time possible and with the minimum possible restriction by staff with the relevant training
- The capacity of the Service User must be considered and action must be taken in line with the five principles of the MCA
- All staff must have PBS training

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8. Key Facts - People Affected by The Service

People affected by this service should be aware of the following:

- You have the right to live your life with freedom, choice and control
- When you become distressed and this presents a risk to you or others, IGNIS CARE LIMITED will ensure that it works within the law to support you safely and effectively while considering your rights at all times. IGNIS CARE LIMITED will always involve you to ask what you would want us to do to support you when you are distressed
- Care Plans are used to enable IGNIS CARE LIMITED to capture your preferences and wishes and provide staff with clear guidance about how they provide you with Support. These Care Plans include your views and wishes that relate to areas that could have the potential to restrict your freedom or choice

Further Reading

MIND - Mental Health Crisis Care: Physical restraint in crisis:

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https://www.mind.org.uk/media-a/4378/physical_restraint_final_web_version.pdf

Beyond Limits:

https://beyondlimits-uk.org/

Restraint Reduction Network:

https://restraintreductionnetwork.org/

RCN "Let's Talk About Restraint" - Rights, Risks and Responsibility:

https://restraintreductionnetwork.org/wp-content/uploads/2016/11/Lets-talk-about-restraint.pdf

BILD:

https://www.bild.org.uk/resources/

Use of Force Act 2018 guidance. This Act is not applicable to supported living services but provides good background information and understanding about the need to reduce the use of force, and how it could be done:

https://www.gov.uk/government/publications/mental-health-units-use-of-force-act-2018/mental-health-units-use-of-force-act-2018-statutory-guidance-for-nhs-organisations-in-england-and-police-forces-in-england-and-wales

NHSE - Identifying restrictive practice weblink:

https://www.england.nhs.uk/long-read/identifying-restrictive-practice/

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Outstanding Practice

To be "outstanding" in this policy area you could provide evidence that:

- All relevant staff are fully aware of the issues surrounding restrictive practices including restraint/physical intervention and understand the implications for themselves and the Service User
- Stakeholders are extremely satisfied with the service. The way in which IGNIS CARE LIMITED manages behaviours that might lead to restrictive practices including restraint/physical intervention is seen as 'best practice'
- Other services have learnt from the way in which IGNIS CARE LIMITED manages behaviour that might lead to restrictive practices
- There are no incidents of restrictive practices including restraint/physical intervention and all staff have training in, and knowledge of, the positive behaviour support approach
- IGNIS CARE LIMITED can demonstrate that it is open and transparent in the area of restrictive practices including restraint/physical intervention, learning from individuals and their relatives or friends about how it can be avoided
- Service Users and their representatives are extremely satisfied with the way that care and support is provided by IGNIS CARE LIMITED
- The wide understanding of the policy is enabled by proactive use of the QCS App

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Forms

The following forms are included as part of this policy:

Title of form	When would the form be used?	Created by
Restraint Physical Intervention Register - CR77	To provide evidence to support practice improvement and reduction of restraint/physical interventions.	QCS
Restraint/Physical Intervention Flow Chart - CR77	To be used to guide decision-making before using any form of restraint.	QCS
Post Restraint/Physical Interventional Analysis - CR77	Post event, this form should be completed as an opportunity for reflective learning and demonstration of continuous improvement.	QCS
Individual Staff Post-incident Debrief - CR77	To debrief after an incident involving restrictive practices.	QCS
Safe Holds Easy Read - CR77	To support enabling service users to understand safe holds.	QCS
Positive Behaviour Support - CR77	A poster that can be displayed to raise awareness.	British Institute of Learning Disabilities (BILD)
Service User Debrief - CR77	This form is to be used following an incident. It should be completed with the service user to aid reflection.	QCS

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Restraint Physical Intervention Register - CR77

Name of Service User	Date, time and duration of restraint/physical intervention	Reason for restraint/physical intervention	Type of restraint/physical intervention	Actions taken as a result of the restraint/physical intervention	Name of person/people involved in the restraint/physical intervention

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Post Restraint/Physical Interventional Analysis - CR77

All staff involved in the incident should attend the post-intervention analysis during which this form is completed.

Person carrying out post-intervention analysis with staff:

Names of staff involved in post restraint/physical intervention analysis:		**Ref number:**	
Attendees at post intervention analysis:		**Date:**	
Name of Service User who had restraint/physical intervention:			
Location, time and date of incident:			
Description of the challenging, concerned, stressed or distressed behaviour leading up to the restraint/physical intervention:			
Who made the decision to physically intervene?			
Staff involved with actual restraint/ physical intervention:			
Did the Service User struggle or become more distressed during the restraint/physical intervention?			
What occurred during the restraint/physical intervention? (Details around de-escalation, any medication used or any concerns during the restraint/physical intervention)			
How long was the person physically restrained for?			
Were any injuries sustained by the Service User or staff?			
What happened after the person was released from restraint/physical intervention?			

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Views and opinions on whether the decision to restrain/physically intervene was necessary, proportionate to the level of risk, or whether a less restrictive option could have been carried out:	
Opinions on whether the incident was well managed:	
Any other issues to be addressed, **e.g. Service User concerns, training**	
Comments of interviewer:	
Have all staff involved received the relevant training and are they up to date with this and deemed competent? (Provide details)	
Further action required:	
Signature of Manager:	
Date:	

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Individual Staff Post-incident Debrief - CR77

Date of restrictive practice/ restraint/physical intervention:		**Reference Number:**	
Brief description of incident:			
Name/post of staff member receiving support:			
Name/post of staff member providing support:			
Were any physical injuries sustained (please tick)?	**Yes**	**No**	
If, 'YES', what injuries?			
What first aid/medical attention was needed?			
Does the staff member require time out and if so, what has been provided?			
Is the staff member able to complete their shift/continue to attend work?			
How is the staff member feeling post incident?			

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Does the staff member require further support and if so, what support?			
Is further debriefing needed for the staff member and/or related to the incident? If it is, please describe.			
What changes can be made to reduce further incidents? e.g. review risks assessments, PBS plans			
Has the staff member identified any further suggestions to reduce risks or personal support needs?			
Date:		**Sent to Manager (Tick to confirm)**	
Signature of staff member receiving support:			
Signature of staff member providing support:			
Further action taken by manager:			
Signature of manager:			
Date:			

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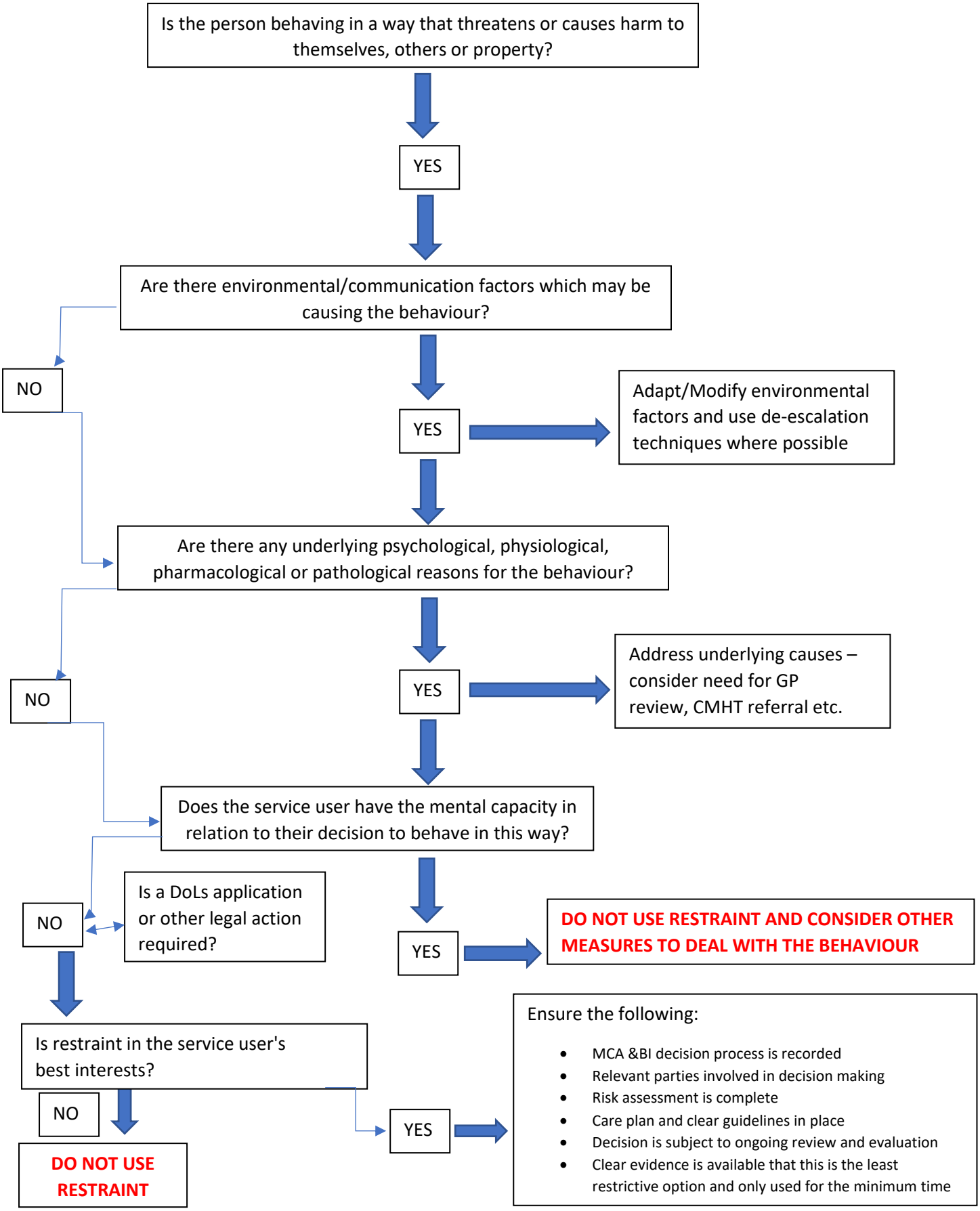
Flow Chart to Guide Decision Making

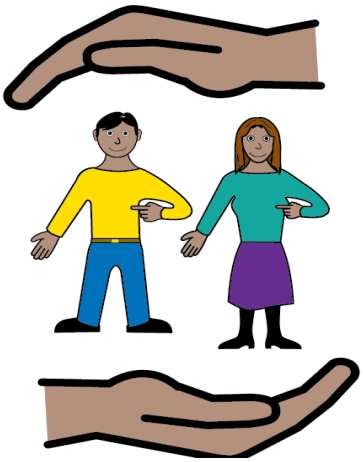


Let’s talk about Safe Holds

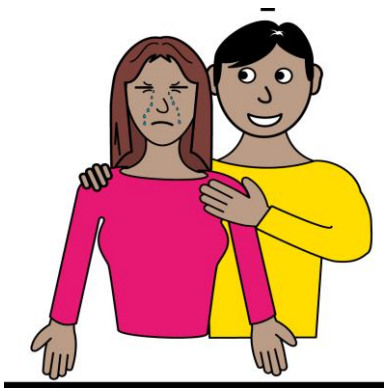
Why would we need to hold someone?

No one should hit another person.

Sometimes a service user may hit out because they may be really angry or upset.



Staff cannot let this happen and must keep everyone safe.



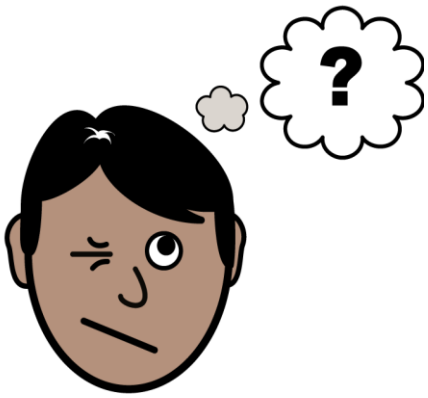
So if a person hits or hurts someone, staff may have to hold that person.

Staff will try and move the person away from others.

We will try and support the person privately away from others.

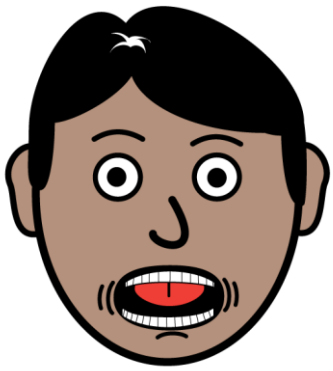
How we will support you

Safe holds will never be used to punish people; only to keep people safe.



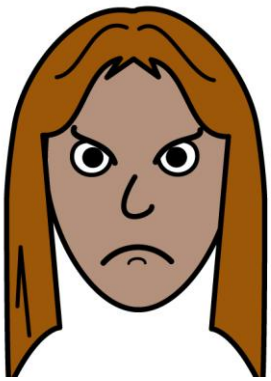
It can be hard to understand what Is happening.

It can be very busy, noisy and scary.



You may feel frightened.

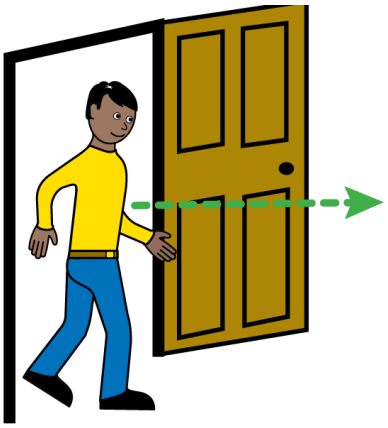
How we will support you



You may feel angry with the person

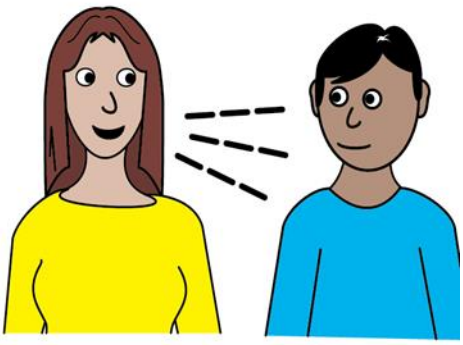
Or

You may feel angry with staff.



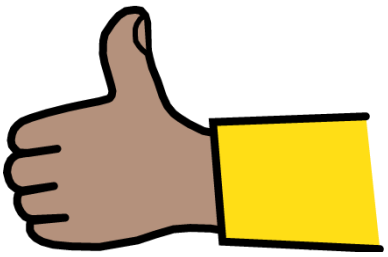
It is best to walk away. No one likes being watched when they are angry or upset.

Remember, staff have training to support people who are upset or angry.



If you are confused, upset or angry, you can talk to a staff member you can trust.

How well do you understand safe holds now?



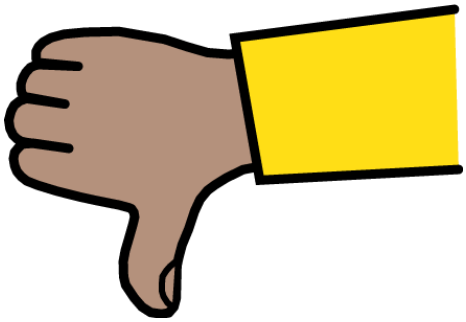


bild.org.uk/uk-pbs-alliance

UK PBS Alliance

POSITIVE BEHAVIOUR SUPPORT

VALUES LED

PBS supports human rights and promotes respect, dignity, inclusion and a life without unnecessary restriction. PBS means treating people equally, celebrating diversity and working in partnership with the person and their family to make things better for everyone. PBS does not advocate the use of punishment or any aversive methods.

PROMOTING QUALITY OF LIFE

The overall aim of PBS is to improve the quality of a person's life and that of the people around them. This includes children, young people and adults, as well as older people. PBS provides the right support at the right time for a person so they can lead a life that is meaningful and interesting to them.

UNDERSTANDING BEHAVIOUR AND MEETING NEEDS

PBS uses different methods to gather information to work out what people's behaviour means if they are unable to express this. It improves support and empowers people to use better and less harmful ways to get their needs met. This often involves using a range of different approaches and personalised ways of supporting that enhance a person's life.

MAKING SYSTEMS WORK FOR THE PERSON

Giving the right support at the right time so people can thrive and fulfil their potential. This may mean changing the way the person is supported. Carers and staff may need training, and service structures and cultures may need to change. Continuous review is important to make sure support carries on working well for the person and those around them. We believe that systems and environments should change not people.

Developed by A MacDonald, E Jones, K Lowe, S Leitch and the UK APBS Alliance, 2019. Updated 2020.

Service User Debrief

How did you feel during the incident?

OK

not sure

upset

scared

How do you feel after the incident?

OK

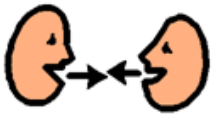
not sure

upset

scared

How could staff support you to feel better?

have a cup of tea


talk to me

go to my room

go for a walk

Is there anything you want to talk about?

☐ yes ☐ no

Staff Member:_____ Service User: _____

Date: _____